



Laboratory Investigation Report

Patient Name	: Mr. Prikshit Prakash	Centre	: 5544 - Visit Health Pvt Ltd -Corporate
Age/Gender	: 45 Y 11 M 10 D / M	OP/IP No/UHID	: //
MaxID/Lab ID	: ML06685472/4828032600149	Collection Date/Time	: 13/Mar/2026 09:15AM
Ref Doctor	: Other	Reporting Date/Time	: 13/Mar/2026 03:06PM

Clinical Biochemistry

Visit _VH00AXI-VP _ Male



SIN No: B2B8896234

Test Name	Result	Unit	Bio Ref Interval
Kidney Function Test (KFT) Profile with Calcium, Uric Acid*, Serum			
Urea calculated(UN_c value)	27.2	mg/dl	19.26 - 49.22
Blood Urea Nitrogen Urease with GLDH	12.71	mg/dL	9.0 - 23.0
Creatinine jaffe,alkaline picrate, kinetic with blank rate co	1.01	mg/dL	0.70 - 1.30
eGFR by MDRD MDRD	79.53	ml/min/1.73 m ²	
eGFR by CKD EPI 2021	92.24		
Bun/Creatinine Ratio Calculated	12.58	ratio	10.0 - 20.0
Uric Acid Uricase,peroxidase	5.7	mg/dl	3.7 - 9.2
Calcium (Total) Arsenazo Colorimetric	9.7	mg/dl	8.7 - 10.4
Sodium IT	145.0	mmol/L	136 - 145
Potassium IT	5.03	mmol/L	3.5 - 5.1
Chloride IT	106	mmol/L	98 - 107
Bicarbonate Colorimetric, PEP-C	28.0	mmol/l	22-29

Ref. Range eGFR - Estimated Glomerular Filtration Rate is calculated by MDRD equation which is most accurate for GFRs $\leq$ 60ml / min /1.73 m².MDRD equation is **used for adult population only**.

Category	Ref Interval (ml / min / 1.73 m ²)	Condition
G1	≥ 90	Normal or High
G2	60 - 89	Mildly Decreased
G3a	45 - 59	Mildly to Moderately Decreased
G3b	30 - 44	Moderately to Severly Decreased
G4	15 - 29	Severly Decreased
G5	< 15	Kidney failure

Test Performed at : 7091 - MAX LAB UDYOG VIHAR PHASE 5 D50, 2nd Floor, Plot No D50, Udyog Vihar, Phase V,

Booking Centre : 5544 - Visit Health Pvt Ltd -Corporate, 237, Okhla Industrial Estate Phase 3, Okhla Phase III, Okhla Industrial Area, 1140845566

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Max Super Speciality Hospital, Saket (West Block), 1, Press Enclave Road, Saket, New Delhi - 110 017, Phone: +91-11-6611 5050

(CIN No.: U85100DL2021PLC381826)

Helpline No. 7982 100 200 www.maxlab.co.in feedback@maxlab.co.in

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Clinical Biochemistry

SIN No: B2B8896234

Visit _ VH00AXI-VP _ Male

Test Name	Result	Unit	Bio Ref Interval
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Kindly correlate with clinical findings

***** End Of Report *****

Dr. Partho Baruah
Technical Head & Principal Consultant
MBBS, DCP (Pathology)
HMC No.- HN 22564

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Immunoassay

Visit_VH00AXI-VP _ Male



SIN No: B2B8896234

Test Name	Result	Unit	Bio Ref Interval
Total-Thyroid Profile (T3,T4 & TSH)*, Serum			
T3 (Total) CLIA	1.42	ng/mL	0.6 - 1.81
T4 (Total) CLIA	8.3	µg/dL	3.2 - 12.6
TSH CLIA	3.307	µIU/mL	0.55 - 4.78

Comment

Parameter	Unit	Cord Blood	Adult	1st Trimester	2nd Trimester	3rd Trimester
TSH	uIU/ml	2.3 - 13.2	0.38 - 5.33	0.1 - 2.5	0.2 - 3.0	0.3 - 3.0

Increased in primary Hypothyroidism.
Decreased in primary Hyperthyroidism

Total Thyroid Profile : (Thyroid Function Test, TFT)

T3 (Total), Triiodothyronine

Increase in Hyperthyroidism, and T3 toxicosis,

Decreased in hypothyroidism, states with decreased TBG, and acute and subacute non thyroidal illness

T4(Total) Thyroxine

Increased in Hyperthyroidism, states with increased TBG, Thyrotoxicosis

Decreased in Hyperthyroidism, states with decreased TBG and Strenuous exercise

TSH, Serum : Thyrotropin(Thyroid Stimulating Hormone)

Increased in primary Hypothyroidism.

Decreased in primary Hyperthyroidism.

Note : TSH levels are subject to circadian variation, reaching peak levels between 2 – 4 am and at a minimum between 6 – 10 pm. The variation is of the order of 50% - 206 %, hence time of the day has influence on the measured serum TSH concentrations.

TSH assay is strandized to the 3rd generation for human TSH.

The Cyclical variations may be quite large; therefore the timing of specimen collection must be strictly controlled.

Advise : Kindly do Thyroid Profile/TSH in morning hours only.

Comment: TSH - Ultrasensitive

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Visit _VH00AXI-VP _ Male

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Kindly correlate with clinical findings

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Clinical Biochemistry

Visit _VH00AXI-VP _ Male



SIN No: B2B8896234

Test Name	Result	Unit	Bio Ref Interval
Amylase, Serum*			
Amylase	126	U/L	30 - 118
Ethylidene BlockedpNPG7			

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Visit _VH00AXI-VP _ Male



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Test Name	Result	Unit	Bio Ref Interval
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HbA1c (Glycated/ Glycosylated Hemoglobin) Test*, EDTA

HPLC

Glycosylated Haemoglobin(Hb A1c) 4.90 % < 5.7

HPLC

Glycosylated Haemoglobin(Hb A1c) IFCC 30.04 mmol/mol < 39.0

Calculated

Average Glucose Value For the Last 3 Months 93.93 mg/dL

Calculated

Average Glucose Value For the Last 3 Months IFCC 5.20 mmol/L

Calculated

Interpretation The following HbA1c ranges recommended by the American Diabetes Association(ADA) may be used as an aid in the diagnosis of diabetes mellitus.

HbA1C(NGSP %)	HbA1C(IFCC mmol/mol)	Suggested Diagnosis
≥ 6.5	≥ 48	Diabetic
5.7 - 6.4	39 - 47	Pre- Diabetic
< 5.7	< 39	Non - Diabetic

HbA1C provides a useful index of average glycaemia over the preceding 6-8 weeks.

It is suggested that HbA1c is measured every 6 months in stable patients, every 3 months in patients with unstable metabolic control and every month in pregnancy.

Increased Glycated hemoglobin is a reflection of Hyperglycemia.



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LDH (Lactate Dehydrogenase) Total , Serum*

LDH	167	U/L	120 - 246
Lactate / NAD			

Fasting Blood Sugar (Glucose) , (FBS)*, Fluoride Plasma

Glucose (Fasting)	82	mg/dl	74 - 99
Hexokinase			

Interpretation A fasting blood sugar level from 100 to 125 mg/dL is considered prediabetes Elevated blood glucose levels are seen in:

Diabetes mellitus, Cushing's disease, Acromegaly

Stress, such as from surgery or trauma. Certain medications, especially [corticosteroids](#)

Decreased blood glucose levels can be due to drug induced, [hypothyroidism](#), [addison](#) (adrenal insufficiency)

Total Iron Binding Capacity (TIBC)*, Serum

Iron	104	µg/dL	65 - 175
Ferrozine			
UIBC	197.0	ug/dL	120 - 470
Colorimetric - Ferene			
Total Iron Binding Capacity	301	ug/dL	250 - 425
Calculated			
Transferrin Saturation	34.55	%	17 - 37
Calculated			

Kindly correlate with clinical findings

*** End Of Report ***



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HMC No.- HN 22564

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Immunoassay

SIN No: B2B8896234

Visit _ VH00AXI-VP _ Male

Test Name	Result	Unit	Bio Ref Interval
PSA (PROSTATE SPECIFIC ANTIGEN) TOTAL*, Serum			
Prostate Specific Antigen CLIA	1.08	ng/mL	0.0 - 4.0

Kindly correlate with clinical findings

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Hematology

Visit_VH00AXI-VP _ Male



SIN No:B2B8896234

Test Name	Result	Unit	Bio Ref Interval
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Complete Haemogram, Peripheral Smear and ESR,EDTA*

Haemoglobin	14.3	g/dl	13.0 - 17.0
SLS-Haemoglobin Method			
Packed Cell, Volume	41.8	%	40-50
Pulse Height Detection Method			
Total Leucocyte Count (TLC)	5.72	10~9/L	4.0-10.0
Flowcytometry method using semiconductor laser			
RBC Count	4.91	10~12/L	4.5-5.5
Hydrodynamic focusing (DC detection)			
MCV	85.1	fL	83-101
Calculated			
MCH	29.2	pg	27-32
Calculated			
MCHC	34.3	g/dl	31.5-34.5
Calculated			
Platelet Count	155	10~9/L	150-410
Hydrodynamic focusing (DC detection)			
MPV	10.9	fl	7.8-11.2
Calculated			
RDW	13.9	%	11.5-14.5
Calculated			

Differential Cell Count

Flowcytometry// Light Microscopy

Neutrophils	45.3	%	40-80
Lymphocytes	41.9	%	20-40
Monocytes	4.9	%	2-10
Eosinophils	7.3	%	1-6
Basophils	0.6	%	0-2

Absolute Leukocyte Count

Calculated from TLC & DLC

Absolute Neutrophil Count	2.59	10~9/L	2.0-7.0
Absolute Lymphocyte Count	2.4	10~9/L	1.0-3.0
Absolute Monocyte Count	0.28	10~9/L	0.2-1.0
Absolute Eosinophil Count	0.42	10~9/L	0.02-0.5
Absolute Basophil Count	0.030	10~9/L	0.02-0.1
ESR (Modified Westergren)	4	mm/hr	<=10

Peripheral Smear Examination

RBC: - Normocytic Normochromic

WBC: - Counts within normal limits but DLC shows increased eosinophil count..

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Hematology

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SIN No: B2B8896234

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Platelet: - Adequate**Impression:** Relative Eosinophilia

Kindly correlate with clinical findings

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Test Name	Result	Unit	Bio Ref Interval
Vitamin B12 (Vit- B12), (Cyanocobalamin)*, Serum			
Vitamin B12 CLIA	206	pg/mL	211 - 911

Interpretation Note:- Vitamin B12 (Cobalamin)

Vitamin B12 is tested for patients with GIT disease, Neurological disease, psychiatric disturbances, malnutrition, alcohol abuse. Increased in chronic renal failure, severe CHF. Decreased in megaloblastic anemia.

Advice: CBC, peripheral smear, serum folate levels, intrinsic factor antibodies (IFA), bone marrow examination, if Vit B12 deficient.

Vitamin D, 25 - Hydroxy Test (Vit. D3)*, Serum

25 Hydroxy, Vitamin D CLIA	15.63	ng/mL	30-100
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Ref Range

Vitamin D Status	25 (OH) Vitamin D Concentration Range (ng/ml)
Sufficiency	30-100
Insufficiency	20-29
Deficiency	<20
Potential Toxicity	>100

Interpretation

Vitamin D toxicity can be due to

1. Use of high doses of vitamin D for prophylaxis or treatment
2. Taking vitamin D supplements with existing health problems such as kidney disease, liver disease, tuberculosis and hyperparathyroidism

Vitamin D deficiency can be due to:

1. Inadequate exposure to sunlight,
2. Diet deficient in vitamin D
3. Malabsorption

Advice: Serum calcium, phosphorus and PTH

Kindly correlate with clinical findings

*** End Of Report ***

Test Performed at : 7091 - MAX LAB UDYOG VIHAR PHASE 5 D50, 2nd Floor, Plot No D50, Udyog Vihar, Phase V,

Booking Centre : 5544 - Visit Health Pvt Ltd -Corporate, 237, Okhla Industrial Estate Phase 3, Okhla Phase III, Okhla Industrial Area, 1140845566

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(CIN No.: U85100DL2021PLC381826)

Helpline No. 7982 100 200 | www.maxlab.co.in | feedback@maxlab.co.in

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**Laboratory Investigation Report**

Patient Name	: Mr. Prikshit Prakash	Centre	: 5544 - Visit Health Pvt Ltd -Corporate
Age/Gender	: 45 Y 11 M 10 D / M	OP/IP No/UHID	: //
MaxID/Lab ID	: ML06685472/4828032600149	Collection Date/Time	: 13/Mar/2026 09:15AM
Ref Doctor	: Other	Reporting Date/Time	: 13/Mar/2026 02:12PM

Immunoassay**Visit _VH00AXI-VP _ Male**

SIN No: B2B8896234

Test Name**Result****Unit****Bio Ref Interval**

Dr. Partho Baruah
Technical Head & Principal Consultant
MBBS, DCP (Pathology)
HMC No.- HN 22564

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MaxID/Lab ID	: ML06685472/4828032600149	Collection Date/Time	: 13/Mar/2026 09:15AM
Ref Doctor	: Other	Reporting Date/Time	: 13/Mar/2026 03:06PM

Clinical Biochemistry

Visit_VH00AXI-VP _ Male



SIN No: B2B8896234

Test Name	Result	Unit	Bio Ref Interval
Lipid Profile, Serum*			
Cholesterol Colorimetri Cholestrol esterse and chole	193	mg/dL	< 200
HDL Cholesterol Accelerator selective detergent methodology	37.0	mg/dL	> 40
LDL Cholesterol Direct	139	mg/dL	< 100
Triglyceride GPO	123.0	mg/dL	30 - 149
VLDL Cholesterol Calculated	24.6	mg/dl	< 30
Total Cholesterol/HDL Ratio Calculated	5.2	..	0.0-4.9
Non-HDL Cholesterol Calculated	156.00	mg/dL	< 130
HDL/LDL Calculated	0.27	Ratio	0.3 - 0.4

Interpretation

Total Cholesterol	Desirable: < 200 mg/dL Borderline High: 200-239 mg/dL High ≥ 240 mg/dL	LDL-C	Optimal: < 100 mg/dL Near Optimal/ Above Optimal: 100-129 mg/dL Borderline High: 130-159 mg/dL High: 160-189 mg/dL Very High: ≥ 190 mg/dL
HDL-C	Low HDL: < 40 mg/dL High HDL: ≥ 60 mg/dL	Triglyceride	Normal: <150 mg/dL Borderline High: 150-199 mg/dL High: 200-499 mg/dL Very High: ≥ 500 mg/dL

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Clinical Biochemistry

Visit_VH00AXI-VP _ Male



SIN No:B2B8896234

Test Name	Result	Unit	Bio Ref Interval
Liver Function Test (LFT)*, Serum			
Total Protein Biuret	7.10	g/dl	5.7 - 8.2
Albumin Bromocresol Green	4.5	g/dL	3.4 - 5.0
Globulin Calculated	2.6	g/dL	2.7 - 4.3
A.G. ratio Calculated	1.7		1.2 - 1.5
Bilirubin (Total) Vanadate oxidation	0.5	mg/dl	0.3 -1.2
Bilirubin (Direct) Vanadate oxidation	0.16	mg/dl	<=0.3
Bilirubin (Indirect) Calculated	0.34	mg/dL	0.1 - 1.0
SGOT- Aspartate Transaminase (AST) Modified IFCC	22	U/l	<34
SGPT- Alanine Transaminase (ALT) Modified IFCC	18	U/L	10 - 49
AST/ALT Ratio Calculated	1.22	Ratio	
Alkaline Phosphatase IFCC Standardization	123	U/L	46 - 116
GGT (Gamma GT), Serum Enzymatic Rate	12.0	U/L	7 - 50

Kindly correlate with clinical findings

*** End Of Report ***



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